

Erythema Dyschromicum Perstans

The disease is characterized by hyperpigmented macules and patches of variable shape and size with an ashen-gray to brown-blue color. They may have a thin, raised erythematous border in the early stage that later can be absent or, mainly in darker skin types, evolve into a hypopigmented border that accentuates the hyperpigmentation. The eruption can also be polymorphic, presenting simultaneously hypo- and hyperpigmented macules. Lesions occur primarily on the face, the neck, the trunk, and the proximal part of the arms. The lesions are usually asymptomatic, although slight pruritus can be reported. There is a slow progression of the lesion over several years, usually without spontaneous regression.

Erythema dyschromicum perstans is mainly observed in intermediate skin types. It is a disease of young adults, although there are small series reporting the disease in prepubertal children. There is no clear sexual predilection.

Histologic examination of the inflammatory border may demonstrate lichenoid dermatitis with vacuolization of the basal cell layer, occasional colloid bodies, and increased epidermal melanin. The dermal changes are edema of the papillary dermis, a moderate or mild lymphohistiocytic infiltrate, and dermal melanophages.

In inactive lesions there is no vacuolization of the basal cell layer, a diminished dermal infiltrate, and an increased number of dermal melanophages. There is no effective treatment for the disease.

Idiopathic Eruptive Macular Hyperpigmentation

Idiopathic eruptive macular hyperpigmentation is an uncommon disorder of pigmentation characterized by an eruption of asymptomatic, brown macules 5 mm to several cm in diameter involving the trunk, neck, and proximal extremities in children and adolescents. There is no previous drug exposure and the lesions disappear gradually over several months to years.

Histopathologically, there is hyperpigmentation of the basal layer of the epidermis and prominent dermal melanophages. The number of mast cells is normal.

Mixed Hypo- and Hyperpigmentation

Dyschromatosis Hereditaria Universalis

Dyschromatosis hereditaria universalis is an autosomal dominant disorder that usually presents in infancy or early childhood in Asian families and is characterized by pinpoint to pea-sized hypo- and hyperpigmented macules, distributed in a reticulated pattern over the trunk, abdomen, and limbs, usually sparing the face and palmo-plantar surfaces.

Reticulate Acropigmentation of Dohi

Reticulate acropigmentation of Dohi is a localized form of dyschromatosis universalis hereditaria, also called dyschromatosis symmetrica hereditaria. It is characterized by symmetric, small, hyper- and hypopigmented macules on the

dorsa of hands and feet and is mainly seen in young children in South American and Asian families.

Westerhof Syndrome

Westerhof syndrome, described in three generations of one family, displays congenital hypo- and hypermelanotic macules. There is also growth and mental retardation.

Vagabond Leukoderma

Vagabond leukoderma is a condition found in persons living in poor hygienic conditions. Many abuse alcohol, live on an inadequate diet, and are infested with lice and/or scabies. Diffuse light-brown hyperpigmentation is present at the shoulder and waist girdle, and the neck and back are dotted with depigmented macules. The condition improves with the institution of a healthier lifestyle and likely represents a co-existence of multiple disorders.